

Policy on Surgical Services

«Hospital Name»

Document control

Document No.	HCO/COP/POL/14	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	OT In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Surgical services are provided consistently and safely, with wrong-site, wrong-patient, and wrong-surgery events prevented through mandatory site marking and time-out, and complete documentation from pre-operative assessment through to the operative note.

1. Purpose

This policy explains how «Hospital Name» provides surgical services under written guidance, assesses patients and obtains consent before surgery, prevents wrong-site, wrong-patient, and wrong-surgery events, documents every operation, plans post-operative care, keeps the operating theatre safe and properly equipped, and runs a quality assurance programme including environmental surveillance.

This policy does not cover general informed consent procedures or infection control in detail — those are covered in other hospital policies. If «Hospital Name» does not provide surgical services, this policy does not apply.

2. Scope

This policy applies to all surgeons, anaesthesiologists, and operating theatre staff involved in surgical services at «Hospital Name».

3. Policy standards

«Hospital Name» provides surgical services under written guidance listing the procedures offered and the competency needed for each, assesses and obtains consent from every

surgical patient, prevents wrong-site, wrong-patient, and wrong-surgery events through mandatory site marking and time-out, documents every operation completely before the patient leaves recovery, plans post-operative care, keeps the operating theatre free of sterile/unsterile mix-ups and properly equipped, and runs a quality assurance programme that includes environmental surveillance of the operating theatre.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not perform a surgical procedure that isn't on the hospital's documented list, or perform it with personnel below the competency level the guidance defines for that procedure.
- Do not proceed with elective surgery without a pre-operative assessment, a documented pre-operative diagnosis, and pre-operative instructions given to the patient and family — do the same for emergency cases whenever possible, and document when it wasn't possible.
- Do not proceed with surgery without informed consent taken by the operating surgeon or a doctor on the operating team.
- Do not perform a new or additional procedure that wasn't planned or consented to, without fresh consent — except where the procedure is life-saving.
- Do not begin a surgical or invasive procedure without completing site marking, where the procedure involves laterality, multiple structures, or multiple levels.
- Do not begin a surgical procedure without holding a time-out immediately beforehand, with every team member present, confirming the correct patient identity, the correct procedure, and the correct surgical site.
- Do not transfer a patient out of recovery before the operative note is completed, covering all eight required elements.
- Do not let an operating theatre run without resuscitation equipment, without instruments in verified working condition, or in a way that mixes sterile and unsterile materials.

5. What we do

5.1 Provide surgical services under written guidance

Written guidance based on standard treatment guidelines or sound clinical practice governs surgical services. It includes the list of surgical procedures «Hospital Name» offers and the competency level required to perform each one.

5.2 Assess and prepare patients before surgery

Every surgical patient has a pre-operative assessment, a documented pre-operative diagnosis, and pre-operative instructions given to everyone concerned, including the patient and family. This applies to all elective cases, and to emergency cases whenever

possible — where it isn't possible for an emergency case, that's documented too. This is done by the operating surgeon or a doctor on the operating team.

5.3 Obtain informed consent

Consent is taken by the operating surgeon or a doctor on the operating team. If the patient's clinical status or expected outcome changes after consent but before surgery, this is explained to the patient and family and documented. If a new or additional procedure comes up that wasn't planned or consented to, fresh consent is obtained — except where the procedure is life-saving, in which case the situation is documented instead. (Related requirements are covered in the hospital's other policies.)

5.4 Prevent wrong-site, wrong-patient, and wrong-surgery events

Written guidance sets out how «Hospital Name» prevents these events — for example, through identification tags, badges, cross-checks, and time-outs.

Site marking is done wherever a procedure involves laterality, multiple structures (like fingers, toes, or lesions), or multiple levels (like the spine). The mark is instantly recognisable, unambiguous, and used the same way throughout the hospital. It's made by the person — or a team member — who will perform the entire procedure and stay with the patient throughout. The patient is actively involved in marking their own site whenever possible. Marking can happen any time before the procedure starts, but the mark stays visible once prepping and draping are complete.

A time-out is held immediately before the procedure starts, with every team member present, to confirm three things together: the correct patient identity, the correct procedure, and the correct surgical site.

5.5 Document the operation

Before the patient is transferred out of recovery, an operative note is completed by the surgeon or a doctor on the operating team. If someone other than the chief surgeon writes it, the chief surgeon countersigns it. At a minimum, it covers eight things: the surgery performed, the name(s) of the surgeon(s), the name(s) of the anaesthesiologist(s), the post-operative diagnosis, any peri-operative complications, the amount of blood loss, the salient steps of the procedure, and the key intra-operative findings.

5.6 Plan post-operative care

Post-operative care follows a documented plan, written by the operating surgeon or a member of the operating team. The plan addresses whatever is clinically relevant — for example, IV fluids, medication, wound care, nursing care, or watching for complications.

5.7 Keep the operating theatre safe from cross-contamination

The layout and practices of the operating theatre prevent sterile and unsterile materials from mixing. It's preferable to use separate closed trolleys for sterile and unsterile goods, though any method that achieves the same outcome is acceptable.

5.8 Keep the operating theatre properly equipped

The operating theatre complex has, at minimum, seven facility areas: a pre-op holding area, changing rooms, hand-washing facilities, operating rooms, a waiting area for relatives, a storage area, and a collection area for waste and linen — plus a recovery room, where applicable. Beyond anaesthesia and surgical equipment, there's resuscitation equipment, and radiation protection equipment where applicable. Instruments are kept in working condition, checked through a defined verification mechanism — feedback from surgeons or anaesthesiologists is one way to do this. Supplies match the scope and complexity of the surgery performed.

5.9 Run a quality assurance programme

The quality assurance programme monitors care-related outcomes — for example, intra-operative mishaps like cautery burns, patient falls, or position-related nerve injuries, and peri-operative events like surgical site infections or deep venous thrombosis. These are examples; «Hospital Name» decides which outcomes matter most for its own surgical practice. The programme also covers pre-operative preparation, antimicrobial prophylaxis, and adherence to procedures that prevent adverse events.

5.10 Monitor the operating theatre environment

The quality assurance programme includes surveillance of the operating theatre environment — daily monitoring of humidity, pressure differential, and temperature, and monitoring of filter integrity at least every six months. «Hospital Name» follows the applicable NABH guidelines for OT air-conditioning.

6. Stop-work authority

Do not begin or continue a surgical or invasive procedure without completed site marking (where laterality, multiple structures, or multiple levels apply) and a completed time-out confirming correct patient, procedure, and site with the whole team present.

Stop-work applies to the start of the procedure. A genuine life-threatening emergency continues with the best available qualified team while the gap is escalated and documented.

The person who stops tells the OT In-Charge and the Medical Superintendent the same shift. Refusing to proceed without site marking or time-out is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
OT In-Charge	Oversees OT operations; ensures written guidance, site-marking and time-out compliance, operative note completion, OT

	safety, equipment readiness, and environmental surveillance are in place; receives stop-work notifications the same shift.
Operating surgeons	Perform or supervise pre-operative assessment; obtain consent (including fresh consent for unplanned procedures except life-saving); perform site marking; hold time-out; complete operative note before patient leaves recovery (eight elements); write post-operative care plan; countersign operative note where applicable.
Anaesthesiologists	Participate in time-out; comply with COP.13 for anaesthesia-specific obligations.
OT nursing and supporting staff	Maintain sterile/unsterile separation; verify instrument working condition; support time-out; assist with daily and periodic environmental monitoring.

8. Quality monitoring

Monitoring area	What is monitored
Written guidance	List of surgical procedures and competency levels documented and current; procedures performed only by staff at the required competency level
Pre-operative assessment	Pre-operative assessment, documented diagnosis, and pre-operative instructions completed for all elective cases; documented for emergency cases when possible, with documented reason when not
Informed consent	Consent taken by operating surgeon or team; fresh consent for new/additional procedures; exception for life-saving procedures documented
Site marking	Site marking completed for all procedures involving laterality, multiple structures, or multiple levels; mark stays visible after draping
Time-out	Time-out held immediately before every procedure with all team members present, confirming correct patient identity, procedure, and surgical site; documented
Operative note	Operative note completed with all eight elements before patient leaves recovery; countersigned by chief surgeon where applicable

Post-operative care plan	Documented post-operative care plan written by operating surgeon or team for every patient
OT sterile/unsterile separation	No mixing of sterile and unsterile materials; defined method in place
OT facility and equipment	All seven facility areas present; resuscitation equipment available; instruments verified working; supplies matched to surgical scope
QA programme — outcomes	Defined care outcomes monitored and analysed (e.g., SSI, DVT, intra-operative mishaps); CAPA applied
Environmental surveillance — daily	Daily monitoring of humidity, pressure differential, and temperature in the OT complex; results recorded
Environmental surveillance — filter integrity	Filter integrity monitored at least every six months; results recorded

9. Training and staff acknowledgement

All surgeons, anaesthesiologists, and operating theatre staff shall be familiar with the written guidance applicable to their work, including pre-operative assessment requirements, informed consent obligations (including the fresh-consent-except-life-saving rule), site marking for laterality/multiple structures/multiple levels, the time-out process, operative note requirements (eight elements), OT sterile/unsterile separation, instrument verification, and environmental monitoring frequencies.

I have read the Policy on Surgical Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all surgeons, anaesthesiologists, and operating theatre staff involved in surgical services at «Hospital Name».

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
DVT	Deep Venous Thrombosis
OT	Operation Theatre
SSI	Surgical Site Infection

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.14.a	Commitment*	Section 5.1 addresses written guidance for surgical services including the list of procedures offered and the competency level required for each procedure.
COP.14.b	Commitment	Section 5.2 addresses pre-operative assessment, documented pre-operative diagnosis, and pre-operative instructions for all elective and, where possible, emergency cases; documents when not possible for emergencies.
COP.14.c	Commitment	Section 5.3 addresses informed consent by operating surgeon or team; documenting changes in patient status after consent; obtaining fresh consent for new or additional unplanned procedures — except where the procedure is life-saving, in which case the situation is documented.
COP.14.d	CORE*	Section 5.4 addresses wrong-site/patient/surgery prevention. Site marking required for three triggers: laterality, multiple structures (e.g., fingers, toes, lesions), and multiple levels (e.g., spine). Mark is recognisable, unambiguous, consistent, made by the procedure performer or a team member who stays with the patient, and remains visible after draping. Time-out held immediately before the procedure with all team

		members present, confirming three things: correct patient identity, correct procedure, and correct surgical site. Stop-work authority (Section 6) also applies to this element.
COP.14.e	Commitment	Section 5.5 addresses the operative note completed before the patient leaves recovery, covering eight elements: surgery performed, surgeon name(s), anaesthesiologist name(s), post-operative diagnosis, peri-operative complications, blood loss, salient steps, and key intra-operative findings. Chief surgeon countersigns if not primary author.
COP.14.f	Commitment	Section 5.6 addresses a documented post-operative care plan written by the operating surgeon or a member of the operating team, covering clinically relevant elements.
COP.14.g	Commitment	Section 5.7 addresses prevention of sterile/unsterile mix-ups in the OT through layout and practices; preferred method is separate closed trolleys, though any equivalent method is acceptable.
COP.14.h	Commitment	Section 5.8 addresses OT complex facility requirements: at minimum seven areas (pre-op holding, changing rooms, hand-washing, operating rooms, relatives waiting, storage, waste and linen collection) plus recovery room where applicable. Also addresses resuscitation equipment, radiation protection where applicable, instrument

		verification, and supply adequacy.
COP.14.i	Achievement*	Section 5.9 addresses the quality assurance programme monitoring care-related outcomes (e.g., SSI, DVT, intra-operative mishaps), pre-operative preparation, antimicrobial prophylaxis, and adherence to adverse-event prevention procedures.
COP.14.j	Achievement*	Section 5.10 addresses OT environmental surveillance at two distinct frequencies: daily monitoring of humidity, pressure differential, and temperature; and monitoring of filter integrity at least every six months. These two frequencies are separate requirements. NABH OT air-conditioning guidelines are followed.

13. Required Records/Evidence Checklist

Written guidance and procedure list

- Written guidance for surgical services including the list of procedures offered and the competency level required for each.

Pre-operative assessment and consent

- Pre-operative assessment, documented diagnosis, and pre-operative instructions for every elective surgical patient.
- Documentation when pre-operative assessment was not possible for an emergency case.
- Informed consent records for every surgical patient, taken by the operating surgeon or team.
- Fresh consent records for unplanned procedures; or documentation of life-saving circumstances where fresh consent was not obtained.

Site marking and time-out

- Site marking records for all procedures involving laterality, multiple structures, or multiple levels; mark documented as visible after draping.

- Time-out records for every procedure, confirming correct patient identity, correct procedure, and correct surgical site with all team members present.

Operative note and post-operative care

- Operative note completed before patient leaves recovery, covering all eight required elements; chief surgeon's countersignature where applicable.
- Documented post-operative care plan for every surgical patient.

OT safety and equipment

- Evidence of sterile/unsterile separation method in use in the OT.
- OT facility checklist confirming all seven areas are present and functional; recovery room where applicable.
- Evidence of resuscitation equipment and radiation protection equipment where applicable.
- Instrument verification records (defined mechanism; working condition confirmed).

Quality assurance and environmental surveillance

- QA programme documentation: defined outcome metrics, monitoring records, and CAPA for surgical care.
- Daily OT environmental monitoring records: humidity, pressure differential, and temperature.
- Filter integrity monitoring records: at least every six months.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.14.
2. Guidebook interpretation supplied for COP.14.a through COP.14.j.
3. NABH guidelines for OT air-conditioning (reference for COP.14.j environmental surveillance).

Disclaimer

This policy reorganises the supplied COP.14 objective elements and approved plain-language content into policy format. The stop-work text — which corresponds to COP.14.d (CORE*) — is reproduced exactly as supplied. Mandatory requirements and their stated modal strength have been retained. The three site-marking triggers (laterality, multiple structures, multiple levels), the time-out's three mandatory agreements (correct patient identity, correct procedure, correct surgical site), the eight operative-note elements, the seven OT facility areas, and the two distinct environmental monitoring frequencies (daily: humidity/pressure differential/temperature; at least every six months: filter integrity) are all preserved exactly and kept separate. The fresh-consent-except-life-saving exception

(COP.14.c) is preserved exactly. The policy is scope-conditional: it does not apply if surgical services are not provided.